

**إسم المريض رابعياً : غاده خليل حلمي فهم**

## الحى / المدينة : امبايه

### المحافضة : الحيز ه

رقم البطاقة : 28806252101241      رقم التلفون : 01206260969

**العمر(تاريخ الميلاد) : 25-06-1988 المهنة : ربه منزل**

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-  
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة  
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه  
التوقيع: (-----)

## التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

## نتائج الأبحاث : (مرفق)

### التشخيص النهائي : سرطان بالثدي (مبادرة)

**خطة العلاج :** أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام

التعليق على التشخيص : -----

GR1-E  
روستاقی کمیٹی  
نئی تحصیل  
ضلع مظفر  
صحة الایات روستا  
اللجنة و مدير

**الاسم: د/محمد کیلانی**

**الاسم: د/ندی حسن**

**الاسم:** د/نرمين رمضان

**الوظيفة: ----- التوقيع:**

**الوظيفة: ----- التوقيع:**

**الوظيفة: ----- التوقيع:**

تعتمد صحة البيانات المرفقة  
رئيس اللجنة ومدير  
المستشفى.

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس**

**وتلتزم جهة العلاج بلائحة المجالس الطبية.**

**كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.**

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

## قرار اللجنة

### التشخيص النهائي :

### خطة العلاج :

### المستشفى :

### جهة العلاج : --

**ملاحظات : ---**

**قسم :**

## نَفَقَات :

## ختم

## شعار الجمهورية

## مدير عام

## مديرية الشئون الصحية

## رئیس

## المجلس الطبي المتخصص

|  |
|--|
|  |
|--|

|                                                                        |                                                                                     |
|------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> غاده خليل حلمي فهمي<br><b>National ID:</b> 28806252101241 |  |
|------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بأن نوضح فيما يلي تقريراً عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد أنه |
|----------------------------------------|

|                                                                                     |
|-------------------------------------------------------------------------------------|
| المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging |
|-------------------------------------------------------------------------------------|

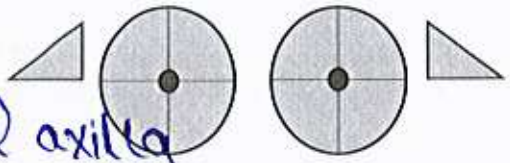
|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |



MDT FORM 22/4/2026 G

|                   |                |                |      |
|-------------------|----------------|----------------|------|
| Name              | غادة خليل حليم | MRN            | ٤٨٦٧ |
| Age               | ٣٧ سنة         | ID             |      |
| Menopausal status | Pre menop.     | Phone          |      |
| NRS SCORE         |                | Marital status |      |
| BMI               |                | Family history |      |
| CO morbidity      |                | REASON OF MDT  |      |

Clinical Data

|                                                       |                                                                                     |            |
|-------------------------------------------------------|-------------------------------------------------------------------------------------|------------|
| BX: IDC G2                                            | R                                                                                   | L          |
| ER: 8/8                                               | PR: 8/8                                                                             | HER 2: -ve |
| Ki67:                                                 |                                                                                     |            |
| Sonomammography:                                      |  |            |
| 10/2024: MBC to contralateral axilla & bone (by E.Ts) | TNM                                                                                 |            |
| Mets Workup: bone scan. → Free                        |                                                                                     |            |
| Started AI + Zoladex + Zometa 13m                     |                                                                                     |            |
| 4/2026: CT: RD of lymph node                          |                                                                                     |            |

Recommendations: SD of Rest

4/2026: bone scan: met. bone newly developed

For 2nd line.

to Zoladex / 13m / 26m

MDT OUTCOME & PLAN

to Zoladex 13m ce. + Zometa 9.3m

SIGNATURE

| MEDICAL ONCOLOGY                                                                   | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | Psychiatric |
|------------------------------------------------------------------------------------|---------|-----------|-----------|-----------|-------------|
|  |         |           |           |           |             |

23/10/2024, 12:54

code 42 -- Name\_A MDT (on Hainnal) -- Name\_E MDT (on Hainnal)



Presidential Program for Women's Health  
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة  
وزارة الصحة والسكان



## MDT form

A-Patient ID:

1- Name: **د. محمد علي محمد**

Sex:

3- Medical number:

Female

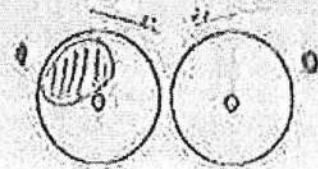
81221055

01/11/2024

23/10/2024

2- ID number:

Patient Age: 36 Y



Family history:

Family history:

Phone:

Tumor:

01002245958

BMI:

HT:

REASON OF NOT

REASON OF NOT

marital status:

Marital status:

NBS Score:

NBS Score:

GROUP:

GROUP:

Date of MDT:

Date of MDT:

Co-morbidity:

Co-morbidity:

Menopausal status:

Menopausal status:

INITIAL STAGE:

INITIAL STAGE:

23/10/2024

## Clinical data

Breast Imaging:

Mets Workup:

Mets status: equivocal. It is no lesion

|                    |             |             |             |             |             |
|--------------------|-------------|-------------|-------------|-------------|-------------|
| DX                 | ER          | PR          | Her 2 Neu   | SISH        | Ki67        |
| IDC                | 8/8         | 8/8         | Her 2 Neu   | SISH        | 10          |
| EXPLANATION        | EXPLANATION | EXPLANATION | EXPLANATION | EXPLANATION | EXPLANATION |
| rt GII & extensive |             |             |             |             |             |

|    |    |                                 |
|----|----|---------------------------------|
| T  | N  | M                               |
| 4b | 3c | 1 bone and contralateral axilla |

Sonomammography

cesm: rt UOQ 9-12 o'clock 52 x22 mm reaching nipple &amp; 6-7 o'clock 6x3.5 mm

+ve axillary LN +ve supracavicular LN

lt level I, II axillary LN --&gt; mets from rt side

rt iliac bone mixed lytic &amp; sclerotic lesion

PATIENT HISTORY

## MDT OUTCOME &amp; PLAN

ONCOLOGY &amp; RX

start 1st line Zoladex + A1, 4 zommetas every 3 months with regular clinical assessment.

| SIGNATURE        |           |  |  |
|------------------|-----------|--|--|
| MEDICAL ONCOLOGY | RADIOLOGY |  |  |
| SURGERY          | PATHOLOGY |  |  |
| Psychiatric      | Nutrition |  |  |

Patient Name : الأستاذة / د. د. خليل حليم فهد  
Patient No. : 28806252101241  
Path No. : 2300/24  
Sex : Female  
Age : 36 Y  
Referring Doc :

Request No. : 13001432  
Request Date : 08/07/2024  
Verification Date : 14/07/2024  
Financial Category : برنامج التأمين - ممرض  
Department : Out Patient  
Registered by : Mrs. Mayada Hegazy

### Pathology Report

Clinical Data: Right breast 10 O'clock mass.  
Nature of Specimen: Tru-cut biopsy, collected at 07/07/2024.

#### Gross Examination:

Multiple grayish white tissue cores up to 1.5 cm long, totally submitted.

#### Microscopic Examination:

Histological type: Invasive duct carcinoma.

Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.

Ductal carcinoma in situ: Extensively detected.

Perineural invasion: Not detected.

Angiolymphatic invasion: Not detected.

#### Diagnosis:

Right breast mass, Tru-cut biopsy:  
Invasive duct carcinoma, Grade II, with extensive DCIS.  
For further IHC (ER, PR, Her2, Ki67).

Signature  
Prof. Dr. Lobna Omar.  
Dr. Esraa Magdy.

Verified by :  
Dr. Duaa Mohamed

Printing Date : 24/07/2024 13:09:54

Page 1 of 1

Printing By : Marwa Mohamed Younis



Patient's Name: Ghada khalil helmy fahmy  
Date: 2026-03-24  
Referred by: Prof.Dr. cap+c  
Hospital: Goustave Roussey International Egypt

**MULTISLICE POST CONTRAST CT SCAN OF THE  
CHEST, ABDOMEN AND PELVIS:**

**TECHNIQUE:** Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

**FINDINGS:**

**Clinically reported right breast carcinoma.**

**Compared to the last study dated 29-11-2025 ,the current study revealed .**

**(I) Chest:**

- Right breast lesion couldn't be appreciated at current study

**Regressive course regarding :**

- Right enlarged axillary lymph nodes , the largest measures 2 x 1.7cm compared to 2.3 x 2 cm
- Enlarged left axillary lymph nodes , the largest 17 x 10 mm compared to 20 x 19mm.
- Bilateral lower lobar mosaic attenuation of small airway disease.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

**(II) Abdomen and pelvis:**

**Patient's Name:** Ghada khalil helmy fahmy  
**Date:** 2026-03-24  
**Referred by:** Prof.Dr. cap+c  
**Hospital:** Goustave Roussey International Egypt

- The liver is enlarged in size showing homogenous CT density no hepatic focal lesions .
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- No worrisome lytic or sclerotic bony lesions ,.
- No vertebral structural collapse .

**OPINION:**

- Regressive course regarding size of bilateral axillary lymphadenopathy .
- Rather stationary course of rest of study

**Dr.Reham Khedr, MD**

Name : Ghada Khalil Helmy Fahim

Date : 23-03-2026 08:43

Branch : Dokki

Medical No. : 334788

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاقد مستشفيات

## ISOTOPIC BONE SCAN REPORT

- **Indication:** 37-year-old female with history of breast carcinoma, presenting with bone aches. Evaluation for possible osseous metastatic disease.
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection of the tracer (Tc-99m MDP).

### FINDINGS:

- Mild abnormal radiotracer uptake is noted along the posterior aspect of the left 10th rib as well as the right sacroiliac region, where metastatic etiology remains a primary consideration.
- No other osseous lesions are identified throughout the remainder of the skeleton.
- Age-related degenerative/arthritis changes are noted in the spine, sternum, both shoulders, and weight-bearing joints.

### IMPRESSION:

- Two worrisome osseous lesions as described, likely metastatic, for radiological correlation and clinical follow-up.
- No other osseous lesions elsewhere.

Much oblige  
Dr. Alaa Nagi, MD  
Nuclear medicine

Patient Name: Ghada Khalil Helmy  
Prof. Dr: Center Hormal  
Accession: 240905004

Patient ID: 684855  
Exam Date: 7/9/2024

### <sup>99m</sup>Tc-MDP whole body bone scan

#### *Desired information:*

- ⊖ Skeletal surveying

#### *Technique:*

- ⊖ Anterior and posterior Delayed Whole body bone scan performed 2 hours after 1. administration of 20 mCi <sup>99m</sup>Tc-MDP;

#### *Report:*

##### *Delayed static phases:*

- ⊖ Active tracer uptake is noted at the left 10<sup>th</sup> rib posteriorly for CT correlation ?? traumatic ?? otherwise.
- ⊖ Mild degenerative changes are noticed in the dorsal & lumbar vertebrae.
- ⊖ Otherwise, no hot spot that account for osseous deposits could be detected all over the rest of surveyed skeleton.
- ⊖ Both kidneys are visualized.

#### CONCLUSION:

- ⊖ Equivocal left 10<sup>th</sup> rib lesion for CT correlation to exclude serious nature.
- ⊖ Follow up bone scan is advised.

Best regards;  
Dr. Huda Fathy M.D.